A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Chris Olszewski
Access HC

Dear Dr Olszewski,

Re: Mr Narayana Quartermain
DOB: 04/10/1978 Phone: 0410 410 114
Address: 48 Carlisle Street, ST KILDA, VIC, 3182
Email: nquartermain@gmail.com

Summary

- **Proctitis in clinical and biochemical remission with a normal faecal calprotectin.**
- **Likely anal fissure and symptomatic haemorrhoids following a period of straining.**
- **Reduction of mesalazine suppositories to three times weekly for maintenance.**
- **No action required by you.**

I had the pleasure of reviewing Narayana Quatermain in the clinic today. You will be pleased to know that his proctitis is in clinical remission. He has completed a course of corticosteroids and has transitioned well to maintenance therapy. He reports that his previous symptoms of urgency, loose stools, and the passage of mucus and blood have entirely resolved.

Biochemically, he is also in remission. His recent faecal calprotectin was 8 mg/kg, which is an excellent result, and his blood tests show no evidence of systemic inflammation and normal haematology. His screening for viral hepatitis was also negative.

His primary concern today relates to perianal discomfort and the recurrence of haemorrhoids. This appears to have been triggered by an episode of significant straining and holding his bowel movements for a prolonged period. He describes discomfort while sitting and a sense of aggravation in the anal canal. Based on the history and a review of clinical photography Narayana provided, I suspect he has developed a small anal fissure in addition to his internal haemorrhoids. This may be further exacerbated by his technique for inserting mesalazine suppositories, as he has been using improvised materials and perhaps inserting them more aggressively than required.

We discussed his other medications, including dexamphetamine for ADHD. I have reassured him that this will not aggravate his proctitis and he may adjust the dose as required under the guidance of his psychiatrist. He continues to avoid non-steroidal anti-inflammatories (NSAIDs), which is appropriate given his history. He has also seen a dietitian and is maintaining an excellent diet; I have advised him that he can reintroduce chilli in moderation as it does not cause colonic inflammation, despite its potential to cause sensory irritation.

Impression and Plan

I have advised Narayana on several conservative measures to manage his perianal symptoms. This includes the use of a footstool to achieve a squatting position during defaecation to reduce pelvic floor pressure, and

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Endoscopy locations
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St Vincent's Private Hospital
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the use of medical-grade lubricant (such as KY Jelly) and gloves for suppository insertion. He does not need to insert the suppositories deeply; they simply need to pass the anal sphincter to reach the rectum.

Regarding his proctitis, we will now de-escalate his therapy. He will reduce the mesalazine suppositories to three times weekly while continuing his mesalazine granules daily. If he remains in remission on this regimen for the next three months, we may trial ceasing the suppositories altogether while maintaining the oral granules.

For the suspected fissure and haemorrhoids, he will increase his use of Rectogesic to a minimum of twice daily for a fortnight. I have also provided a backup prescription for a compounded nifedipine and lignocaine cream to be used if his symptoms do not settle with conservative measures. We discussed the option of rubber band ligation for his haemorrhoids during his next surveillance procedure, but we have decided to defer this for now in favour of conservative management to avoid the associated post-procedural pain and bleeding risks.

I will review Narayana in six months with a repeat faecal calprotectin. Should he experience any symptoms of a clinical relapse, I have asked him to contact me sooner for an earlier review and consideration of therapy escalation.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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